

Matrix of Australian frameworks, governance and compliance tools to guide the responsible and transparent use of AI/ML in healthcare research

Ranked by relevance to health researchers and clinicians involved in advanced analytics and AI in health, most to least relevant, grouped in four tiers.

Authors: A/Prof Michael Guihot, QUT Law School, Dr Bernadette Hyland-Wood, QUT Centre for Data Science.

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Key	
Resource	Document name and originating institution.
Status	Whether the instrument binds, e.g., law (mandatory), funding condition, regulator guidance, framework, voluntary.
Licensing	(a) open and freely reusable; (b) open with attribution requirements; (c) requires permission or negotiation; (d) requires procurement or legal review. Licensing classifies reuse of the document itself (e.g. in teaching or internal guidance), not the data or regime it governs; Creative Commons (CC) licences exclude Australian Government coats of arms, logos and third-party content.
Applies to	Who it binds, e.g., Commonwealth (Cth) agencies, private sector, researchers, HRECs, institutions
Lifecycle stage	where it fits in a research project (ethics approval → data access & linkage → data management → model development → validation & trials → deployment).
Why it matters	Short description of relevance to health research and clinical delivery
Currency	Last updated (as of June 2026) and anticipated revision schedule.
Constraints & watch points	Constraints mean <i>must comply</i> and are established by legislation, regulatory instruments, or institutional policy and are typically non-negotiable. Failure to meet a constraint may result in ethics approval being withheld or revoked, data access being denied, legal liability, or professional sanctions. Watch points mean <i>assess applicability, monitor for change</i>. Watch points are governance considerations that are emerging, conditional, or context-dependent. A watch point may not apply to every project, but researchers should assess whether it applies to theirs, and should be prepared for it to become a constraint as the regulatory landscape evolves.

#	Resource	Status	Licensing	Applies to	Lifecycle stage	Why it matters for health data research	Currency (June 2026)	Constraints / watch points
TIER 1 — Core research ethics and health data access (gatekeeping instruments every project meets)								
1.	NHMRC National Statement on Ethical Conduct in Human Research (2025)	Mandatory funding/HREC condition	(b) CC BY 4.0 (NHMRC)	Researchers, HRECs, institutions	Ethics approval; all stages	The threshold instrument for any research using human data; the 2025 update strengthens treatment of data, databanks and emerging technologies.	Updated 2025; current	Not AI-specific; HREC capability on AI varies widely
2.	Privacy Act 1988 (Cth) and the Australian Privacy Principles in the Schedule to the Act	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	Cth agencies; private sector incl. private health	Data access; data management; model development	Governs collection, use and disclosure of health information ('sensitive information') by Cth-regulated entities, incl. research re-use.	Under continuing reform	Does not cover state public hospitals; AI re-use strains APP 3 and APP 6
3.	Privacy Act s 95 / 95A / 95AA Guidelines	Binding statutory guidelines	(b) CC BY 4.0 (NHMRC)	HRECs, researchers, data custodians	Ethics approval; data access	The legal mechanism by which HRECs authorise use of identifiable health information without consent — the workhorse of secondary-use research.	Stable; review likely with Tranche 2	Cth-regulated data only; state Acts have parallel waiver provisions
4.	State/territory health privacy laws — e.g. Information Privacy Act 2009 (Qld); Health Records and Information Privacy Act 2002 (NSW); Health Records Act 2001 (Vic)	Binding law (state/territory)	(b) CC BY 4.0 on state legislation sites — eg Qld Legislation - verify per jurisdiction	Public hospitals, state agencies, some private providers	Data access; linkage	Most Australian clinical data sits in state public systems governed by these Acts, not the Commonwealth Privacy Act 1988 — the first question for any data access request.	Varies; Qld amended 2025 (incl. Mandatory Notification of Data Breach scheme)	Fragmented; multi-site projects face inconsistent rules across jurisdictions

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5.	Australian Code for the Responsible Conduct of Research (2018) + supporting guides (same site)	Mandatory funding condition	(b) CC BY 4.0 (NHMRC/ARC/UA)	Researchers, institutions	All stages	Research-integrity baseline; data management, authorship and peer-review guides apply directly to AI-assisted research.	Stable; guides periodically updated	Principles-based; institutional AI-use policies still catching up
6.	NHMRC Ethical Conduct in Research with Aboriginal and Torres Strait Islander Peoples (2018) + Keeping Research on Track II	Mandatory funding/HREC condition	(b) CC BY 4.0 (NHMRC)	Researchers, HRECs	Ethics approval; all stages	Required for any research involving Indigenous peoples or their data; six core values applied by HRECs.	Established	Must be applied through community governance, not as a checklist
7.	NHMRC Management of Data and Information in Research (2019)	Guide supporting the Code	(b) CC BY 4.0 (NHMRC)	Researchers, institutions	Data management	The most direct national guidance on retention, sharing and re-use of research data.	Stable	Pre-dates generative AI
8.	AIATSIS Code of Ethics (2020)	Mandatory for AIATSIS-funded; strong normative force generally	(c) © AIATSIS — free to access; reuse beyond fair dealing needs permission	Researchers	Ethics approval	Indigenous research ethics including data and digital materials; complements the NHMRC guidelines.	Established	Context-specific application required
9.	Maiam nayri Wingara Indigenous Data Sovereignty principles	Normative	(c) No formal licence — engage Maiam nayri Wingara / Indigenous governance	Researchers, data custodians	Data access; data management	Indigenous data sovereignty and governance expectations for any dataset containing Indigenous data — increasingly expected by custodians and funders.	Growing adoption	Not codified in legislation; requires genuine partnership structures
10.	Framework for Governance of Indigenous Data (NIAA 2024)	Australian Government guidance	(b) CC BY 4.0	Australian Government (APS) agencies	Whole data lifecycle	FAIR + CARE principles, ethical collection and disaggregated access	First published May 2024. Still current	Guidance only — no enforcement mechanism
11.	Privacy and Other Legislation Amendment Act 2024 (Tranche 1)	Binding law - amends the <i>Privacy Act 1988</i> (Cth) Will expire but amendments will remain in the Privacy Act	(b) CC BY 4.0 (Federal Register of Legislation)	All APP entities	Data management; deployment	Statutory tort for serious invasions of privacy (June 2025) raises the stakes of data misuse; ADM transparency obligations commence Dec 2026.	Enacted; staged commencement	Tranche 2 (fair-and-reasonable test, consent reform) still outstanding
12.	Data Availability and Transparency Act 2022 (DATA Scheme)	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	ONDC, Cth custodians, accredited users	Data access; linkage	The accreditation pathway for researchers seeking Commonwealth datasets (e.g. MBS/PBS) for linked-data research.	Active scheme	Cth data only; accreditation effort; some uses excluded
13.	Five Safes Framework (ABS / AIHW)	Governance model (sector-adopted)	(b) CC BY 4.0 (ABS); AIHW materials vary	Data custodians, linkage units, researchers	Data access	The de facto risk framework every Australian custodian applies when assessing access requests — know it before you apply.	Embedded in custodian practice	Not law; applied with varying stringency
14.	My Health Records Act 2012 + Framework for Secondary Use of MHR Data	Binding law + policy framework	(b) CC BY 4.0 (legislation; Dept of Health framework)	Researchers; AIHW as data custodian	Data access	Governs research access to the national EHR and the conditions of secondary use.	Pathway still maturing	Opt-out dynamics affect representativeness; access remains limited
15.	Healthcare Identifiers Act 2010	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	Providers, linkage units	Linkage	Regulates use of healthcare identifiers (IHIs) in data linkage.	Stable	Narrow scope

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16.	ARDC Research Data Management Framework (FAIR; institutional RDM)	Sector framework	(b) CC BY 4.0 (ARDC)	Universities, researchers	Data management	National infrastructure and practice baseline for managing and sharing research data.	Maintained	Requires institutional adoption; pair FAIR with CARE for Indigenous data
17.	OAIC guidance on AI and privacy	Regulator guidance	(b) CC BY 4.0 (OAIC)	APP entities, researchers	Model development; deployment	The regulator's position on applying the APPs to AI development and to use of commercial AI tools with personal information.	Current	Guidance, not law; oriented to commercial tools
TIER 2 — Clinical AI, translation and liability (when the research builds, tests or deploys an AI tool)								
18.	Therapeutic Goods Act 1989 + TGA guidance on AI/ML medical devices (incl. Clinical Trial Notification Clinical Trial Approval pathways)	Binding law + regulatory interpretation	(b) CC BY 4.0 (legislation 4.0; TGA site CC BY 3.0 AU)	Sponsors, developers, researchers	Validation & trials; deployment	Determines whether an AI tool is a regulated medical device and which clinical trial pathway applies — the gateway for translational AI.	Actively maintained	Software boundary (clinical decision support exemptions) contested
19.	National Clinical Trials Governance Framework (ACSQHC)	National framework (accreditation)	(b)/(c) CC BY-NC-ND 4.0 (ACSQHC) — exact copies only; adaptation needs permission	Health service organisations	Validation & trials	Site-level governance and accreditation for trials, including clinical AI trials.	Current	Site-level focus; sponsor obligations sit elsewhere
20.	Australian Commission on Safety and Quality in Health Care ACSQHC AI guidance , incl. Pragmatic AI Guidance for Clinicians (2025)	National guidance	(b)/(c) CC BY-NC-ND 4.0 (ACSQHC) — exact copies only; adaptation needs permission	Health services, clinicians	Deployment	Safety-and-quality expectations when AI tools enter clinical care — the deployment benchmark for translational projects.	Current	Deployment-focused, not design-stage
21.	AHPRA / National Boards guidance on Meeting your professional obligations when using Artificial Intelligence in healthcare (2025)	Regulator guidance	(c) © AHPRA — personal/organisational use permitted; otherwise permission	Registered health practitioners	Deployment	Professional obligations when practitioners use AI; directly relevant to clinician-researchers and embedded studies.	Current	Practice-focused
22.	Professional college AI standards (e.g. RANZCR Standards of Practice for AI; RACGP; ACRRM)	Professional standards	(c) © colleges — some resources member-gated; reuse needs permission	Clinician-researchers	Validation; deployment	Discipline-specific expectations (radiology AI, AI scribes) that shape study design and clinician participation.	Actively developed	Uneven coverage across disciplines
23.	Federal anti-discrimination law (Racial Discrimination Act 1975, Sex Discrimination Act 1984, Disability Discrimination Act 1992, Age Discrimination Act 2004)	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	All entities	Model development; deployment	The primary legal hook for algorithmic bias in health AI — central to fairness analyses and bias-liability questions.	Stable; technology-neutral	No AI-specific provisions; complaint-based enforcement

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24.	AHRC Human Rights and Technology report (2021) + subsequent AI work	Commission recommendations	(b) CC BY 3.0 (AHRC)	Government, organisations	Model development	Bias, transparency and human-rights framing for automated decision-making in Australia.	Foundational; partially actioned	Not binding
25.	Australian Consumer Law (CCA Sch 2)	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	Suppliers, developers	Deployment	Misleading conduct in AI performance claims; product liability for software outside the TGA perimeter.	Stable; ACCC active on digital markets	Not health- or AI-specific
26.	NHMRC Statement on Consumer and Community Involvement in Health and Medical Research (2026)	National guidance	(b) CC BY 4.0 (NHMRC)	Researchers, institutions	Design; ethics approval	Funders and HRECs increasingly expect co-design, including of AI tools and data governance.	Established	Aspirational; uneven institutional uptake
TIER 3 — AI governance frameworks and policy direction (context and partner obligations)								
27.	National AI Centre - Guidance for AI Adoption — Foundations & Implementation (Oct 2025)	Voluntary national guidance	(b) CC BY 4.0 (DISR/NAIC)	All organisations	Model development; deployment	Now the primary Commonwealth guidance (the AI6 practices); superseded the Voluntary AI Safety Standard.	Current	Voluntary; generic rather than health-specific
28.	Australia's AI Ethics Principles (2019 – updated 2025)	Voluntary policy	(b) CC BY 4.0 (DISR)	All sectors	All stages	The normative baseline cross-referenced by nearly all later Australian AI guidance.	Stable	High-level; voluntary
29.	National Framework for the Assurance of AI in Government (2024)	Cross-jurisdictional framework	(b) CC BY 4.0 (DTA/Finance)	Government agencies	Deployment (public sector)	Assurance expectations for AI used by public health bodies and government research partners.	Stable, foundational	Requires local implementation
30.	AI Policy: Responsible Use of AI in Government v2.0	Mandatory (Cth agencies)	(b) CC BY 4.0 (DTA)	Cth agencies	Deployment (Cth)	Binds Commonwealth health bodies participating in or hosting research.	Versioned, actively maintained	Cth government only
31.	Data Governance Framework (2025)	Mandatory (Cth agencies)	(b) CC BY 4.0 (Finance)	Cth agencies	Data management (Cth)	Data quality and re-use standards relevant when partnering with Commonwealth custodians.	Recently updated	Not AI-specific
32.	Qld AI Governance Policy / QGEA	Mandatory (Qld Government)	(b) CC BY 4.0 (Qld Government)	Qld public sector	Deployment (Qld)	Binds Queensland Health and HHS partners — the operative AI rules for most Qld-based health data research.	Updated	Jurisdiction-specific; check equivalents in other states
33.	Use of generative AI in Queensland Government (2025)	Government guidance	(b) CC BY 4.0 (Qld Government)	Qld public servants	Tool use	Day-to-day GenAI use rules applying to Queensland Health staff collaborating on research.	Current	Tool-use focused
34.	National AI Plan (Dec 2025)	Commonwealth policy	(b) CC BY 4.0 (DISR)	Government, industry, research	Strategic context	Confirms reliance on existing laws plus voluntary guidance; shelved the proposed mandatory guardrails; established the AI Safety Institute.	Current; signals future direction	Not binding; no operational detail
35.	Australian AI Safety Institute	Commonwealth institution	(b) CC BY 4.0	Government, regulators, researchers	Horizon scanning	Shapes future AI risk-assessment expectations through analysing and testing new AI models, support regulators in responding, shape safe AI development and deployment, monitoring and gap analysis.	New	Advisory only; no enforcement powers

TIER 4 — Adjacent and situational (relevant to specific projects or institutional settings)								
36.	<u>Copyright Act 1968</u> / IP framework	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	Developers, researchers	Model development	Training data, text-and-data mining and ownership of AI outputs when building models on clinical corpora.	Under review (AI and copyright)	No TDM exception; output ownership unsettled
37.	<u>ISO/IEC 42001 (AI management systems)</u> ; NIST AI RMF	Voluntary international standards	(d) <u>ISO/IEC 42001: purchase (ISO / Standards Australia)</u> : \$242.18 AUD (a) NIST AI RMF: US public domain	Institutions, research organisations	Assurance	Increasingly requested in collaborations, grants and procurement as assurance evidence.	Maintained	Not Australian law; certification cost
38.	<u>EU GDPR + EU AI Act</u> (extraterritorial reach)	Binding where an EU nexus exists	(b) EUR-Lex — reuse permitted with source acknowledgment	International / multi-site collaborations	Multi-site research	Applies where EU partners or EU data subjects are involved; AI Act obligations phase in to 2027.	AI Act phasing; Digital Omnibus may amend	EU nexus only; compliance complexity
39.	<u>ACSC Essential Eight</u>	National guidance	(b) CC BY 4.0 (cyber.gov.au)	All organisations	Data security	Baseline cyber controls custodians expect of institutions holding sensitive health data.	Updated periodically	Baseline only
40.	<u>Security of Critical Infrastructure Act 2018</u> (SOCI)	Binding law	(b) CC BY 4.0 (Federal Register of Legislation)	Hospitals and health systems as critical infrastructure	Data security (institutional)	Security obligations on the health systems hosting research data and AI infrastructure.	Active regime	Institutional, not researcher-level, obligations
41.	<u>Digital Health Strategy</u> (2023–28)	National strategy	(b) CC BY-NC 4.0 (Dept of Health / ADHA)	Health sector	Strategic context	System-level context for interoperability and data infrastructure underpinning research.	Ongoing	Strategic, not operational